

PSYCHOSOCIAL HAZARD MAPPING — SWITCHBOARD SERVICES

Prepared under the Work Health and Safety Act 2011 (Qld), the Work Health and Safety

Regulation 2011 (Qld), and the Managing the risk of psychosocial hazards at work Code of

Practice 2022

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August 2026 — prepared for the risk assessment under clause 7.2 of the certified agreement, and for consultation under sections 47 to 49 of the Act. Companion to "Return to Work and Workplace Controls — Proposals for Consultation". Nothing in this document is an allegation against any person; every hazard is described as a feature of the work and its systems.

PART 1 — THE FRAMEWORK THIS MAPPING SITS IN

1. Section 19 of the Act places the primary duty on the person conducting the business to ensure, so far as is reasonably practicable, the health of workers — health is defined to include psychological health.
2. The Regulation requires psychosocial risks to be managed: hazards identified, risks assessed, controls implemented in the order of the hierarchy, and controls reviewed — including when a hazard is identified or on request.
3. Sections 47 to 49 require consultation with the workers who are, or are likely to be, directly affected — at each step: identification, assessment, control selection, and review.
4. The Code of Practice 2022 identifies the recognised psychosocial hazard categories and the data sources for assessing them, including complaints records, absence and turnover data, and workload data. This mapping uses the Code's categories and terminology.

PART 2 — THE MAPPING: HAZARD, PRESENTATION, RISK, PROPOSED CONTROL

1. **HIGH JOB DEMANDS.** How it presents: sustained call volume (in the order of 200 to 300 calls per shift), a substantial proportion from distressed or urgent callers; concurrent operation of multiple systems; emergency code activations through the shift; additional functions absorbed when other support is unavailable. Risk: cognitive overload, sustained hyper-vigilance, exhaustion. Proposed controls: evidence-based rostering matched to demand peaks (Proposal 7); defined escalation and backup pathways so absorbed functions return to their owners (Proposals 1 and 4 of the operating model).
2. **LOW JOB CONTROL.** How it presents: operational changes taking effect without consultation with the operators who work the board; no operator input into rostering design. Risk: helplessness in a high-demand role — the demand/control combination the Code identifies as high risk. Proposed controls: consultation before operational change (Proposal 2); operator consultation in roster design (Proposal 7).
3. **POOR SUPPORT.** How it presents: supervision and decision-support not reliably available in real time, particularly after hours and on weekends, in a room whose work is continuous. Risk: unsupported decision-making in safety-critical moments; accumulated strain. Proposed controls: the rotational integrated manager in hours (Proposal 3); the 24/7 on-shift supervisor function — authority follows the clock (Proposal 6).
4. **LOW ROLE CLARITY.** How it presents: functions accreting to the Switchboard without documented boundaries (complaint handling, departmental support, roster administration); uncertainty as to what is, and is not, the operator's responsibility. Risk: role overload and blame absorption for matters outside the role. Proposed controls: documented boundaries and escalation paths (operating model Proposals 1 and 4); the complaint-response standard returning escalated matters to the accountable function.
5. **POOR ORGANISATIONAL CHANGE MANAGEMENT.** How it presents: changes to processes and system access made without consultation with affected operators, with downstream effects discovered in

operation. Risk: preventable failures experienced at the console; erosion of trust. Proposed control: consultation before operational change (Proposal 2), applied systematically as the Act already requires.

6. INADEQUATE REWARD AND RECOGNITION. How it presents: no development pathway within the room; competence and additional responsibility carrying no recognition in classification or progression. Risk: effort-reward imbalance, disengagement, loss of experienced staff. Proposed control: the rotating on-shift supervisor function with in-charge recognition and a trained pathway (Proposal 6).

7. POOR ORGANISATIONAL JUSTICE. How it presents: inconsistency between the timeframes and process standards applied to operators and those observed in matters affecting them; complaints and workplace matters raised without acknowledgment, outcome, or feedback. Risk: the perception and experience of unfairness — a recognised psychosocial hazard in its own right; escalating distress in those affected. Proposed controls: the complaint-response standard (Proposal 4); one rulebook for the room (Proposal 5); written feedback on WHS matters raised (Part 1, item 7 of the companion document).

8. REMOTE OR ISOLATED WORK (AFTER HOURS). How it presents: after-hours and weekend shifts performed without present supervision or management support, while the room's functions continue unchanged. Risk: unsupported exposure to all other hazards during the periods of least support. Proposed control: the 24/7 on-shift supervisor function (Proposal 6).

9. EXPOSURE TO DISTRESSED AND AGGRESSIVE CALLERS. How it presents: the Switchboard is the hospital's first point of contact; a substantial proportion of contacts are made in distress, fear, conflict or crisis, and complaints are frequently expressed as accusations to the operator answering. Risk: cumulative exposure to occupational violence and aggression in verbal form. Proposed controls: the complaint-response standard and escalation pathway so the operator's exposure ends at triage (Proposals 1 and 4); acknowledgment of this exposure in the assessment's scope.

10. FATIGUE. How it presents: rostering patterns capable of producing breaks below the mandated minimum; recovery dependent on the operator's own leave. Risk: impaired performance in a safety-critical function; cumulative harm. Proposed control: fatigue rules built into roster design — minimum breaks enforced structurally, not checked after the fact (Proposal 7).

PART 3 — CONSULTATION AND REVIEW

1. This mapping is offered as a worker's contribution to hazard identification under sections 47 to 49 — the first step of the process the Regulation requires. It is not a substitute for the assessment; it is an input to it.

2. I ask that the assessment: adopt the Code's categories; draw on the Code's data sources (complaints records, absence and turnover data, EAP usage, workload and call-volume data); be conducted by an assessor independent of the functions being assessed; and select controls in consultation, in the order the Regulation requires.

3. I ask that controls, once implemented, carry review dates, and that the review include the operators affected — consistent with the Regulation's review triggers.

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